

Alcohol-related Liver Disease (ALD) — Clinical One-Pager

Facilitator quick reference · TeachIM (Mowry/Haydek, Nov 2022), optimized Jun 2026 · CC BY-NC 4.0 · clinician-review before use

Spectrum: hepatic steatosis → steatohepatitis (ASH) → fibrosis → cirrhosis. Steatosis & inflammation regress with abstinence; **fibrosis is largely irreversible.**

Stages & Risk Factors

Stage	Features / reversibility
Steatosis	~90% of heavy drinkers; benign marker; reversible
Steatohepatitis (ASH)	Inflammatory; indolent or acute AH; infl. reversible, fibrosis not
Cirrhosis	End-stage fibrosis; irreversible

Risk: ≥ 2 drinks/d (F), ≥ 3 /d (M); bingeing; genetics; obesity; female sex; smoking; concurrent liver dz.

Diagnostic Evaluation

Screen all adults >18 w/ unhealthy use (CAGE, AUDIT-C).

Initial: RUQ US + CMP, CBC, INR.

Entity	Key features
Steatosis	Incidental; LFTs nl/mild $\uparrow$; indistinguishable from MASLD → history clinches it
AH	Clinical dx; AST:ALT >2 , both <400 ; $\uparrow$ Tbili
Fibrosis/cirrhosis	FIB-4, APRI, FibroTest, ELF; elastography/MRI; nodular liver

AST/ALT >400 – 500 → not pure AH: r/o ischemic, APAP, viral.

Management — 4 Pillars

Pillar	Specifics
Abstinence (#1 survival)	Naltrexone PO/IM (hepatic; caution adv dz) or acamprosate PO TID (avoid eGFR<30); AVOID disulfiram ; + psychosocial/MAT
Nutrition	High-protein diet; thiamine/folate/B-complex; low-Na if portal HTN; consider zinc in AH
Complications	Ascites/SBP, varices (EGD), HE, HCC surveillance, viral hep vacc/screen
Transplant	Refer non-recovering cirrhosis; consider early LT for select severe AH non-responders

Acute Alcohol-related Hepatitis (AH)

Dx (AASLD clinical): heavy use (F ≥ 3 / M ≥ 4 drinks/d $\times \geq 60$ d), jaundice ≤ 8 wk, AST:ALT >2 (both <400 IU/L), Tbili >3 mg/dL. Often $\uparrow$ WBC/PMN, RUQ pain. **R/o infection in EVERY pt.**

Severity: MDF (Tbili+INR) ≥ 32 or MELD >20 = severe (30–50% 30-d mortality).

Steroids: prednisolone **40 mg/d** if no contraindication; **contraindic:** active infxn, AKI Cr >2.5 , uncontrolled GIB, MOF/shock, alt cause. $\pm$ IV NAC (short-term survival, 2024 ACG).

Lille (day 4–7)	Action
<0.45 responder	Complete 28-d course (~85% 6-mo survival)
≥ 0.45 non-resp	Stop steroids → palliative / LT eval

Pearls & 2024 Updates

- Abstinence beats every drug; at steatosis the liver can fully recover.
- Steroids in AH buy **weeks, not years**, and only in Lille responders.
- 2024 ACG ALD guideline** reaffirms MDF ≥ 32 / MELD >20 + Lille rule; pred+NAC adjunct; early LT for select severe AH.
- 2023 nomenclature:** NAFLD → MASLD, NASH → MASH; MetALD = metabolic dysfunction + moderate alcohol.
- STOPAH (2015): pentoxifylline no benefit; prednisolone short-term only.